

LANDING PAGE AUDIT

Meridian Physio — new patient landing page

The page reads as a practice brochure, not as an answer to the question the visitor arrived with. Most of the recoverable enquiries are lost in the first screen and in the form — both are cheap to change. The measurement problem is listed second because until it is fixed, nothing else can be judged.

PREPARED FOR Meridian Physio (specimen)	PAGE AUDITED <code>https://example.com/meridian-physio/new-patient</code>
CONVERSION GOAL A completed new-patient booking enquiry submitted through the form on the page.	DATE 8 September 2026
FINDINGS 8 ranked	METHODOLOGY Rubric v1 · Template 1.0.0

Do this week

The three highest-priority findings, with the cheapest change that moves each one. Reasoning, evidence and the remaining findings follow.

1

The headline describes the practice, not the visitor's problem

QUICK FIX

Replace the headline with the visitor's problem and the outcome. Three to test, in preference order: "Shoulder, knee or back pain? Assessed and treated this week." / "See a physio within 48 hours — most patients start treatment at the first visit." / "Get a clear diagnosis and a plan at your first appointment." Move "evidence-based" to a supporting line where it does useful work.

CRITICAL

QUICK · UNDER 2 HOURS

STRUCTURAL

2

Enquiries are counted on page view, not on submission

QUICK FIX

Add a submit-listener event on the form so that a genuine submission is recorded separately from the thank-you page view. Keep both events for a fortnight so the gap between them is quantified.

HIGH

MEDIUM · HALF A DAY TO TWO DAYS

BEHAVIORAL

3

"Submit" does not say what the visitor gets

QUICK FIX

Rewrite the four pieces of copy as one promise. Heading: "Book your first appointment". Hint: "Tell us what is going on and we will call you back the same business day with a time." Button: "Request my appointment". Delete the fine print.

HIGH

QUICK · UNDER 2 HOURS

STRUCTURAL

How to read this report. Findings are ranked in the order we would fix them, not by how easy they are. Severity states how much the finding is costing the page; effort states what it takes to change. 5 further findings are set out in the priority table overleaf. This report diagnoses and sequences; it does not predict a conversion rate.

SECTION 2

Priority table

All findings in the order we would address them. Business impact is the consequence of leaving the finding in place.

#	FINDING	PILLAR	SEVERITY	EFFORT	BUSINESS IMPACT IF UNADDRESSED
1	The headline describes the practice, not the visitor's problem headline-outcome--9dc6f1fd	Structural	CRITICAL	Quick under 2 hours	The most expensive traffic on the site — paid search on high-intent injury terms — lands on a page that does not confirm the match. Enquiries are lost before any of the practice's actual advantages are read.
2	Enquiries are counted on page view, not on submission analytics-integrity--0d2adf65	Behavioral	HIGH	Medium half a day to two days	Every decision about this page is being made against a number that is inflated by an unknown amount. Campaign optimisation, budget allocation and any assessment of the changes in this report all inherit that error. This is listed second because it is the finding that makes the others measurable.
3	"Submit" does not say what the visitor gets primary-cta-clarity--2252ea92	Structural	HIGH	Quick under 2 hours	The action with the highest intent on the page is described in the vaguest language on the page. It costs nothing to fix and is being paid for on every click.
4	The form asks for seven fields to arrange a phone call form-friction--83db29a5	Structural	HIGH	Medium half a day to two days	Fields that exist to save reception a question are being paid for in lost enquiries. Referral source in particular is being collected badly here and is available accurately from the ad platform.
5	A quarter of paid spend is on terms this page cannot serve traffic-quality--3660315a	Pre-Build	MODERATE	Medium half a day to two days	Roughly a quarter of the monthly ad spend cannot convert on this page regardless of what the page says. Left in place, it also depresses the measured conversion rate and makes every change look less effective than it is.
6	Price is never stated, and it is the first thing this buyer asks objection-coverage--c84dadf4	Structural	MODERATE	Medium half a day to two days	The practice is filtering out price-sensitive enquiries by attrition rather than by choice, and is spending callback time on questions the page could have answered.
7	On mobile the argument is buried under the form mobile-rendering--eb7cc529	Structural	MODERATE	Quick under 2 hours	The page is being judged on its desktop layout while three quarters of the traffic sees a different and worse one.
8	"Exercise prescription" is not read as treatment comprehension-gap--3660315a	User Testing	LOW	Quick under 2 hours	The practice's actual method is being described in language that undersells it. Cheap to correct, but low priority against the four findings above.

Findings

Finding 1 · headline-outcome--9dc6f1fd

The headline describes the practice, not the visitor's problem

CRITICAL

QUICK · UNDER 2 HOURS

STRUCTURAL

HERO H1

Assessed against: Does the headline state an outcome the visitor wants, in their words?

OBSERVATION

The first line on the page is "Evidence-based physiotherapy, delivered with care". It states a property of the practice. The supporting line — "a modern practice combining hands-on treatment with exercise prescription and a patient-first philosophy" — restates the same property in more words.

Visitors arrive from paid search on injury-specific terms: shoulder pain, knee rehab, back pain. Nothing in the first screen names an injury, a timeframe, or an outcome. The words the visitor typed do not appear anywhere above the fold.

EVIDENCE

SCREENSHOT First screen at 1280px. The headline and subheading both describe the practice; no injury, outcome or timeframe appears. (desktop 1280x820) See appendix.

METRIC **Enquiry rate:** 1.22% (51 enquiries / 4,180 sessions) (1 – 31 August 2026)

Source: Client-supplied GA4 export

Consequence for the visitor

A visitor searching for help with a specific injury cannot tell in the first two seconds whether this clinic treats it. They have two or three other tabs open with the same question, and no reason to prefer this one.

Consequence for the business

The most expensive traffic on the site — paid search on high-intent injury terms — lands on a page that does not confirm the match. Enquiries are lost before any of the practice's actual advantages are read.

RECOMMENDATIONS

QUICK FIX

Replace the headline with the visitor's problem and the outcome. Three to test, in preference order: "Shoulder, knee or back pain? Assessed and treated this week." / "See a physio within 48 hours — most patients start treatment at the first visit." / "Get a clear diagnosis and a plan at your first appointment." Move "evidence-based" to a supporting line where it does useful work.

MEDIUM CHANGE

Split the page by injury group so that the headline matches the ad group that sent the visitor. Three variants (shoulder, knee, back) covers the majority of paid spend.

LONG-TERM REDESIGN

Build headline selection into the campaign structure so every ad group lands on copy that repeats its own search term.

Enquiries are counted on page view, not on submission

HIGH

MEDIUM · HALF A DAY TO TWO DAYS

BEHAVIORAL

BOOKING FORM SUBMIT EVENT

Assessed against: Is the conversion actually being measured, correctly, and attributable to a source?

OBSERVATION

The conversion event fires on load of the thank-you page. That page is directly reachable, is indexed, and receives its own organic traffic. The 51 "form submissions" in the August export therefore include visits that never submitted anything.

No submit-event is recorded on the form itself, so there is no way to separate the two, and no way to tell how many submissions failed validation.

EVIDENCE

OBSERVED The thank-you URL returns 200 and renders fully when requested directly, with no referrer or session check.

Source: Direct request during audit, 4 September 2026

METRIC Form submissions (as reported): 51 (1 – 31 August 2026)

Source: Client-supplied GA4 export — conversion event fires on thank-you page view

Consequence for the visitor

None directly. The visitor is unaffected.

Consequence for the business

Every decision about this page is being made against a number that is inflated by an unknown amount. Campaign optimisation, budget allocation and any assessment of the changes in this report all inherit that error. This is listed second because it is the finding that makes the others measurable.

RECOMMENDATIONS

QUICK FIX

Add a submit-listener event on the form so that a genuine submission is recorded separately from the thank-you page view. Keep both events for a fortnight so the gap between them is quantified.

MEDIUM CHANGE

Make the thank-you page unreachable without a submission token and mark it noindex. Add a failed-validation event so abandoned submissions are visible.

LONG-TERM REDESIGN

Reconcile enquiries against bookings actually made in the practice management system, so the page is measured against revenue rather than against form fills.

"Submit" does not say what the visitor gets

HIGH

QUICK · UNDER 2 HOURS

STRUCTURAL

HERO CTA BUTTON

Assessed against: Is there one primary action, and is what happens after clicking it unambiguous?

OBSERVATION

The form is headed "Get in touch", the hint reads "a member of our team will be in contact", the button reads "Submit", and the closing line reads "We respond to all enquiries."

Across four pieces of copy, the page never states what happens next or when. A visitor with a painful shoulder is trying to work out whether they will be seen this week; the page answers a different question.

EVIDENCE

SCREENSHOT Form heading, hint, button and fine print. None of the four states an outcome or a timeframe. (desktop 1280x820) See appendix.

Consequence for the visitor

The visitor cannot tell whether submitting the form books an appointment or joins a queue for a callback at an unknown time. That ambiguity is a reason to keep looking.

Consequence for the business

The action with the highest intent on the page is described in the vaguest language on the page. It costs nothing to fix and is being paid for on every click.

RECOMMENDATIONS

QUICK FIX

Rewrite the four pieces of copy as one promise. Heading: "Book your first appointment". Hint: "Tell us what is going on and we will call you back the same business day with a time." Button: "Request my appointment". Delete the fine print.

MEDIUM CHANGE

State the actual next available appointment on the page and keep it current from the booking system.

The form asks for seven fields to arrange a phone call

HIGH

MEDIUM · HALF A DAY TO TWO DAYS

STRUCTURAL

#BOOKING-FORM

Assessed against: Does the form ask only for what is needed to deliver the next step?

OBSERVATION

The form collects full name, email, phone, preferred practitioner, health fund, referral source and a free-text description. Only name and one contact method are needed to make the callback that the form promises.

Three of the seven are asked before the visitor has any relationship with the practice: preferred practitioner (they do not know the practitioners), health fund (an administrative question), and referral source (a marketing question, asked of the visitor rather than measured).

EVIDENCE

SCREENSHOT At 390px the form runs well past the first screen; the submit button is not reachable without scrolling through all seven fields. (mobile 390x844) See appendix.

METRIC **Mobile share of sessions:** 71% (1 – 31 August 2026)
Source: Client-supplied GA4 export

Consequence for the visitor
Seven fields on a phone, three of which the visitor cannot answer confidently, is enough friction to defer the enquiry — and a deferred enquiry from a person in pain usually goes to whichever clinic asked for less.

Consequence for the business
Fields that exist to save reception a question are being paid for in lost enquiries. Referral source in particular is being collected badly here and is available accurately from the ad platform.

RECOMMENDATIONS

QUICK FIX

Cut the form to name, phone, and "what is going on?". Delete preferred practitioner and referral source outright; both are better handled on the callback or in the ad platform.

MEDIUM CHANGE

Move health fund to the confirmation step, after the appointment is agreed, where it is an administrative detail rather than a barrier.

LONG-TERM REDESIGN

Replace the enquiry form with direct online booking against real availability. That removes the callback step the form exists to arrange.

A quarter of paid spend is on terms this page cannot serve

MODERATE

MEDIUM · HALF A DAY TO TWO DAYS

PRE-BUILD

Assessed against: Is the page underperforming, or is it being sent the wrong visitors?

OBSERVATION

The August search terms report shows a material share of spend on queries about services the practice does not offer, and on informational queries — exercises, stretches, self-treatment — where the searcher is not looking for a clinic at all.

This matters for how the rest of this report is read: part of the low enquiry rate is not a page problem. Redesigning the page will not recover a click from someone who wanted a stretching video.

EVIDENCE

METRIC Paid search spend: A\$3,400/month (August 2026)
Source: Client-supplied Google Ads summary

OBSERVED Informational and out-of-scope service queries account for roughly a quarter of August spend in the supplied search terms report.
Source: Client-supplied Google Ads search terms report, August 2026

Consequence for the visitor	Consequence for the business
A visitor looking for exercises to do at home lands on a booking page and leaves. Nothing on the page could have helped them.	Roughly a quarter of the monthly ad spend cannot convert on this page regardless of what the page says. Left in place, it also depresses the measured conversion rate and makes every change look less effective than it is.

RECOMMENDATIONS

QUICK FIX

Add negative keywords for the informational and out-of-scope terms visible in the August search terms report. This is a change in the ad account, not on the page.

MEDIUM CHANGE

Segment the reporting so paid enquiry rate is measured only against in-scope terms, and this page is judged on the traffic it is built for.

LONG-TERM REDESIGN

If the informational traffic is worth keeping, give it a destination of its own rather than pointing it at the booking page.

Price is never stated, and it is the first thing this buyer asks

MODERATE

MEDIUM · HALF A DAY TO TWO DAYS

STRUCTURAL

PRICING SECTION

Assessed against: Are the objections that actually stop this buyer addressed on the page?

OBSERVATION

The page carries a "HICAPS on site" badge and nothing else about cost. There is no initial consultation fee, no out-of-pocket estimate, and no statement about what a health fund typically covers.

For a first-time patient choosing between clinics, cost and rebate are among the first two or three questions. The page defers both to the callback.

EVIDENCE

SCREENSHOT The badge row is the only reference to cost anywhere in the first screen. (desktop 1280x820) See appendix.

Consequence for the visitor

A visitor who needs to know whether a first appointment is \$90 or \$180 has to submit a form and wait for a call to find out. Some will; most will check a competitor who published the number.

Consequence for the business

The practice is filtering out price-sensitive enquiries by attrition rather than by choice, and is spending callback time on questions the page could have answered.

RECOMMENDATIONS

QUICK FIX

State the initial consultation fee and a typical out-of-pocket after a standard health fund rebate, in one line under the form.

MEDIUM CHANGE

Add a short fees section covering initial and subsequent appointments, rebates and what is included in the first visit.

LONG-TERM REDESIGN

If price is a genuine competitive weakness, address it with what the fee buys — longer initial appointment, same-day imaging referral — rather than by omitting it.

On mobile the argument is buried under the form

Moderate

Quick · Under 2 hours

Structural

Hero, 390px viewport

Assessed against: Does the page hold its argument and its tap targets at mobile widths?

Observation

At 390px the two-column layout stacks, putting the headline, subheading and four badges above the form. The visitor scrolls through all of it before reaching a single field, and past all seven fields before reaching the button.

The navigation is hidden at this width, so there is no path to fees, team or contact either. Seventy-one per cent of sessions see this version of the page.

Evidence

Screenshot At 390px the form begins below the badge row; the submit button sits several screens down. (mobile 390x844) See appendix.

Metric **Mobile share of sessions:** 71% (1 – 31 August 2026)
Source: Client-supplied GA4 export

Consequence for the visitor
The majority of visitors meet a wall of practice description before anything they can act on, on the device where scrolling costs the most.

Consequence for the business
The page is being judged on its desktop layout while three quarters of the traffic sees a different and worse one.

Recommendations

Quick fix

Cut the badge row to two items at mobile widths and add a sticky call button so the primary action is always one tap away.

Medium change

Reorder the mobile stack: headline, one line of proof, then the shortened form. Move the remaining practice description below it.

"Exercise prescription" is not read as treatment

LOW QUICK · UNDER 2 HOURS USER TESTING

Assessed against: Do first-time readers describe the offer the way the page intends?

OBSERVATION

Read cold, the subheading's "exercise prescription" is clinical vocabulary. To a first-time patient it reads as being sent home with a sheet of exercises rather than being treated — which is close to the opposite of what the practice means by it.

The same applies to "patient-first philosophy", which carries no information a competitor could not also claim.

EVIDENCE

OBSERVED Read against the assumed-knowledge and comprehension checks in rubric v1: both phrases require the reader to already hold the practice's own framing.

Source: Cold read during audit, 4 September 2026

Consequence for the visitor

A visitor in pain reads the one differentiator on the page as a reason to expect less hands-on treatment, not more.

Consequence for the business

The practice's actual method is being described in language that undersells it. Cheap to correct, but low priority against the four findings above.

RECOMMENDATIONS

QUICK FIX

Say it in the patient's words: "hands-on treatment, plus a home programme so the problem does not come back". Delete "patient-first philosophy".

Methodology

What was assessed

- One landing page or one conversion flow, at the URL recorded on the cover.
- The four rubric pillars: Structural, Behavioral, Pre-Build and User Testing.
- Desktop and mobile rendering of the page as served at the time of audit.
- Any analytics or metrics supplied by the client, taken as given and cited where used.

What was not assessed

- Pages other than the one recorded on the cover.
- Implementation of any recommendation in this report.
- A/B test design or execution.
- Metrics the client did not supply and that are not observable from the page itself.
- Back-end systems, payment processing, or anything past the point of conversion.
- This is a specimen report produced to demonstrate the deliverable. Meridian Physio is not a real practice, the page audited is a mock, and every metric shown is illustrative rather than measured.

Findings are ranked against the rubric named above, not against a statistical model of this page's visitors.

Where a finding rests on a client-supplied metric, the metric is cited and the finding inherits its accuracy.

This report makes no prediction of conversion rate change. Impact is stated as a mechanism, not a percentage.

PILLARS APPLIED

Structural	What the page is made of and how it is put together: information hierarchy, offer clarity, message match against the inbound source, form and call-to-action construction, technical delivery (load, layout stability, mobile rendering).
Behavioral	How visitors actually move through the page: attention order, scroll depth, friction and hesitation points, drop-off against the stated conversion goal.
Pre-Build	Decisions made before the page existed that the page is now paying for: audience definition, traffic quality and intent, the offer itself, and whether the conversion goal is the right one.
User Testing	What real people report when asked to use the page: comprehension, stated objections, and the gap between what the page assumes a visitor knows and what they actually know.

Rubric scales

The enumerated values behind every finding in this report, as defined in rubric v1. They are printed in full so that a finding can be re-read, and re-argued, against the frame it was produced under.

SEVERITY

CRITICAL	Prevents conversion outright for an identifiable segment of visitors, or actively misleads them. The page cannot do its job until this is fixed.
HIGH	Suppresses conversion materially. Visitors can still convert, but a substantial share will not because of this. Evidence points to a repeated, not incidental, effect.
MODERATE	Adds avoidable friction or ambiguity. The effect is real but narrower — confined to a segment, a device class, or a single step.
LOW	A defect of craft or consistency with no demonstrated conversion cost. Included because it is cheap to fix, not because it is holding the page back.

EFFORT

Quick under 2 hours	A copy, config or styling change. No new design, no new component, no release risk.
Medium half a day to two days	A layout or component change, or a change touching a form or tracking. Needs design or dev attention but not a rethink.
Large more than two days	A structural rework — new sections, a changed flow, or a decision that reaches back past the page itself. Sequence it, do not squeeze it in.

Appendix — evidence

METRICS

MEASURE	VALUE	PERIOD	SOURCE	USED IN
Sessions	4,180	1 Aug – 31 Aug 2026	Client-supplied GA4 export	Supplied at intake
Form submissions	51	1 Aug – 31 Aug 2026	Client-supplied GA4 export	Supplied at intake
Enquiry rate	1.22%	1 Aug – 31 Aug 2026	Derived from client-supplied GA4 export	Supplied at intake
Mobile share of sessions	71%	1 Aug – 31 Aug 2026	Client-supplied GA4 export	Supplied at intake
Paid search spend	A\$3,400/month	August 2026	Client-supplied Google Ads summary	Supplied at intake traffic-quality- -3660315a
Enquiry rate	1.22% (51 enquiries / 4,180 sessions)	1 – 31 August 2026	Client-supplied GA4 export	headline-outcome- -9dc6f1fd
Form submissions (as reported)	51	1 – 31 August 2026	Client-supplied GA4 export — conversion event fires on thank-you page view	analytics-integrity- -0d2adf65
Mobile share of sessions	71%	1 – 31 August 2026	Client-supplied GA4 export	form-friction- -83db29a5 mobile-rendering-- eb7cc529

SCREENSHOTS

Exhibit 1 · desktop 1280x820 · headline-outcome--9dc6f1fd, primary-cta-clarity--2252ea92, objection-coverage--c84dadf4

Meridian Physio

ServicesOur teamFeesContact

Evidence-based physiotherapy, delivered with care

A modern practice combining hands-on treatment with exercise prescription and a patient-first philosophy.

Est. 2014

APA members

HICAPS on site

Parking available

Get in touch

Complete the form and a member of our team will be in contact.

Full name

Email

Phone

Preferred practitioner

No preference

Health fund

Referral source

Please select

How can we help?

Submit

We respond to all enquiries.

First screen at 1280px. The headline and subheading both describe the practice; no injury, outcome or timeframe appears.

Meridian Physio

Evidence-based physiotherapy, delivered with care

A modern practice combining hands-on treatment with exercise prescription and a patient-first philosophy.

Est. 2014

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At 390px the form runs well past the first screen; the submit button is not reachable without scrolling through all seven fields.